

1. Administrative & Study Identification

Full Study Title: A Phase 3, Randomized, Open-Label Study of Nivolumab Combined With Cabozantinib Versus Sunitinib in Participants with Previously Untreated, Advanced or Metastatic Renal Cell Carcinoma **Brief Study Title:** A Study of Nivolumab Combined With Cabozantinib Compared to Sunitinib in Previously Untreated Advanced or Metastatic Renal Cell Carcinoma **Short Title/Acronym:** CheckMate 9ER (CA209-9ER) **Protocol Number:** CA209-9ER **NCT Number:** NCT03141177 **Posting ID:** 806600574934296172 **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Sponsor/Company Name:** Bristol Myers Squibb **Investigational Product:** Nivolumab (Opdivo) combined with Cabozantinib (Cabometyx) **Active Comparator:** Sunitinib (Trade Name: Sutent | ATC Code: L01EX01 | EMA URL: [Product Information](#)) **Phase of Development:** Phase 3 (Phase Requirement: Trial must be in PHASE3) **Source Level:** 5 **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the Progression-Free Survival (PFS) of Nivolumab combined with Cabozantinib compared to Sunitinib in previously untreated advanced or metastatic Renal Cell Carcinoma (RCC). **Secondary Objectives:** To evaluate Overall Survival (OS), Objective Response Rate (ORR), and the safety and tolerability (number of participants experiencing adverse events) of the treatment regimens.

2.2 Background & Rationale To address the need for additional, highly effective first-line treatment options for patients with advanced or metastatic RCC. Despite early detection and surgical options for small tumors, the mortality rate for advanced disease remains high. This study aims to determine whether combining a PD-1 immune checkpoint inhibitor (Nivolumab) with a multikinase inhibitor (Cabozantinib) is safe and effective compared to the standard-of-care single-agent tyrosine kinase inhibitor, Sunitinib, in delaying disease progression and improving overall survival.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Sunitinib) **Blinding:** Open-label (None) **Randomization:** Randomized (1:1 ratio)

3.2 Planned Enrollment & Duration Targeted Enrollment: 701 randomized participants **Number of Sites:** Global, multi-center
Estimated Study Start: 23-Jul-2017 **Estimated Primary Completion:** 12-Feb-2020

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histological confirmation of RCC with a clear-cell component, including participants who may also have sarcomatoid features. 2. Advanced (not amenable to curative surgery or radiation therapy) or metastatic (AJCC Stage IV) RCC. 3. No prior systemic therapy for RCC (with the exception of one prior adjuvant or neoadjuvant therapy for completely resectable RCC if such therapy did not include an agent targeting VEGF or VEGF receptors and if recurrence occurred at least 6 months after the last dose of adjuvant or neoadjuvant therapy).

4.2 Exclusion Criteria 1. Any active CNS metastases. 2. Any active, known, or suspected autoimmune disease. 3. Any condition requiring systemic treatment with either corticosteroids (> 10 mg daily prednisone equivalent) or other immunosuppressive medications within 14 days of randomization. 4. Participants who have received a live/attenuated vaccine within 30 days of first treatment. *(Note: Other protocol defined inclusion/exclusion criteria could apply. For more information regarding Bristol Myers Squibb Clinical Trial participation, please visit www.BMSStudyConnect.com)*

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Nivolumab 240 mg every 2 weeks (or 480 mg every 4 weeks) in combination with Cabozantinib 40 mg once daily. **Comparator:** Sunitinib (Sutent) **Route of Administration:** Intravenous infusion (Nivolumab) and Oral (Cabozantinib/Sunitinib). **Duration of Treatment:** Until disease progression or unacceptable toxicity (Nivolumab may be given for a maximum of 2 years).

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for co-morbidities. **Prohibited:** Concurrent systemic corticosteroids (> 10 mg daily prednisone equivalent), immunosuppressive agents, and live/attenuated vaccines within 30 days of first treatment.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Baseline Imaging, Safety Labs Baseline Day 0 Randomization, First Dose Administration Interim Q2W /

Q4W | Treatment administration (IV/Oral), Safety Labs, Tumor Imaging | | **End of Study** | At progression/toxicity | Final Vital Signs, Exit Interview, Safety and Survival Follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS), defined as the time from the date of randomization to the first documented tumor progression date or death due to any cause, whichever occurs first. Participants who die without a reported progression will be considered to have progressed on the date of their death. Participants who did not progress or die will be censored on the date of their last evaluable tumor assessment on or prior to initiation of subsequent anti-cancer therapy. Progressive disease (PD) is characterized by a 20% increase in the sum of diameters of target lesions, taking as reference the smallest sum on study. **Measurement Method:** Evaluated by Blinded Independent Central Review (BICR) using RECIST v1.1 criteria.

7.2 Secondary & Exploratory Endpoints 1. Overall Survival (OS), defined as time from randomization to death due to any cause. 2. Objective Response Rate (ORR). 3. Number of Participants Experiencing Adverse Events (AEs).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of treatment-emergent AEs (e.g., diarrhea, fatigue, hepatotoxicity, hypertension) graded by standard CTCAE criteria. **Serious Adverse Events (SAEs):** Evaluated at every contact, with serious events reported rapidly to the sponsor and regulatory authorities. **Data Monitoring Committee (DMC):** Independent safety monitoring committee to oversee patient safety and ongoing efficacy profiles throughout the trial.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all primary and secondary efficacy evaluations (PFS, OS, ORR); Safety Set for all participants receiving ≥ 1 dose of study drug. **Sample Size Justification:** Appropriately powered to detect a statistically significant hazard ratio improvement (target HR ~ 0.60 to 0.70) in PFS and OS for the combination arm vs. Sunitinib. **Handling of Missing Data:** Time-to-event variables will use Kaplan-Meier methodology, with censoring at the date of last contact (last known alive date) or last evaluable tumor assessment for patients without an event.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring visits to ensure protocol adherence and patient safety. **Audit Trail:**

Validated eCRF systems, secure data clarification form logs, and controlled audit trails for all central imaging (BICR) data uploads.

11. Study Identifiers & Governance

Primary ID: CA209-9ER **Registry Number:** NCT03141177 **Posting ID:** 806600574934296172 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** CheckMate 9ER **Official Regulatory Title:** A Phase 3, Randomized, Open-Label Study of Nivolumab Combined With Cabozantinib Versus Sunitinib in Participants with Previously Untreated, Advanced or Metastatic Renal Cell Carcinoma

12. Clinical Framework (Renal Cell Carcinoma Specific)

Primary Condition/Disease Area: Advanced or Metastatic Renal Cell Carcinoma (aRCC) **Preferred UMLS Name:** Renal Cell Carcinoma

Diagnosis Threshold: Histological confirmation of clear-cell RCC, AJCC Stage IV **Medical Methodology:** Immuno-oncology plus Tyrosine Kinase Inhibitor (IO+TKI) combination therapy vs. TKI monotherapy

Optimization Target: Progression-Free Survival and Overall Survival prolongation

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical pathway for first-line advanced RCC treatment **Education Protocol (HCP):** Protocol specific training on Immune-Related Adverse Events (irAEs) and Tyrosine Kinase Inhibitor toxicity management

Education Protocol (Patient): Treatment schedule adherence, AE reporting (e.g., fatigue, diarrhea), and infusion timelines

Quality Audit Mechanism: Source data verification and Blinded Independent Central Review (BICR) for standardized imaging evaluation

14. Observation & Follow-Up Schedule

Total Duration: Follow-up for survival (Estimated up to 4+ years of data tracking) **Onsite Visit Cadence:** Every 2 to 4 weeks corresponding to infusion cycles

Remote Monitoring Frequency: Survival and safety follow-up contacts (e.g., every 3 months post-treatment)

Checklist Review Interval: Radiographic tumor assessments every 8 to 12 weeks

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	06-Apr-2026					
Clinical Operations Lead	[Name]	_____	06-Apr-2026					Lead
Statistician	[Name]	_____	06-Apr-2026					